

could see only shadows. On Oct. 4th the outer and upper fourth of the cornea was clear; the rest was densely leucomatous. There was a large symblepharon from the lower corneal margin to the lower fornix. The United States Golf Association has issued a warning against the dangerous practice of cutting open golf balls, and a Bill has been introduced into the legislature of the State of Massachusetts to prohibit the sale of golf balls containing any caustic fluid.

ACTINOMYCOSIS AND PUBLIC HEALTH.

AN entirely new phase in the history of actinomycosis is now being entered upon as the result of the investigations and conclusions of the medical officers of health of the City and Port of London.¹ These officers find that there is a very considerable traffic in ox tongues from the Argentine Republic, and that a somewhat large proportion of these, about 5 per cent., are affected with actinomycosis either in the blades or in the attached glands. They conclude, therefore, that until action is taken in the Argentine which will ensure that these tongues are carefully examined and the affected ones withdrawn, steps must be taken in the interests of the public health to inspect and regulate this traffic. Though the granules so characteristic of actinomycotic pus were known to Langenbeck and Lebert, and were carefully described by Robin in 1871 as occurring in certain "abcès anciens," it was not until the investigations of Rivolta, Perroncito, and Bollinger during the years 1868-77 on the disease as met with in cattle, that it was recognised as of parasitic nature, and several more years elapsed before the identity of the disease as met with in men and cattle was postulated by Israël and Ponfick. This identity is, however, not admitted by all observers, and there are still lacunæ in our knowledge which, as is suggested in the report we are considering, require to be filled by further investigations. There are certainly a number of varieties of streptothrix found in diseases clinically resembling actinomycosis, some of which are aerobes and some anaerobes, but whether all cases of true actinomycosis in man are caused by the anaerobic form, or whether this holds for the majority of cases in certain localities only, and whether the forms of streptothrix found on fodder grasses are identical with those producing the disease in man and cattle—a convenient and natural assumption—these still remain subjects for investigation. Harbitz and Gröndahl² regard the aerobic forms cultivated from plants as differing from the forms found in human disease, and they could not infect experimental animals with these forms. It is certain that human actinomycosis is not the rare disease which at one time it was thought to be, and is met with fairly often in hospital practice, though, owing to the lack of characteristic symptoms, the cases usually masquerade under many diagnoses before their true nature is settled by bacteriological examination. The grounds on which the imported tongues are to be condemned is that they are diseased; the report is careful to state that there is no proof that there is danger of infection, or any evidence that the disease spreads from cattle to man, or even among cattle themselves. There was also

no proof or suggestion that the organisms found in the tongues were living. The occurrence of the disease, usually in relation to the gastro-intestinal tract, would naturally cause suspicion to attach to food as a means of infection. Attention is drawn to the difficulty of distinguishing in some cases between the lesions of actinomycosis and tubercle; the search for tubercle bacilli in these cases gave no positive results. The relation of actinomycosis to tuberculosis is, of course, intimate and of great interest, and has been insisted upon and elaborated by Mr. A. G. R. Foulerton in his recent Milroy lectures before the Royal College of Physicians of London.³ It is partly on account of this connexion, but chiefly because a large percentage of the tongues are affected in the glands alone, that the report suggests a regulation rendering tongues without roots and glands liable to re-exportation. This question of actinomycosis in cattle is obviously entering upon a very interesting and important phase.

THE OCCURRENCE OF UNILATERAL VISUAL HALLUCINATIONS AND MACROPSIA IN A CASE OF CEREBRAL TUMOUR.

Dr. Arnold Josefson, of Stockholm, has published in a recent number of the *Deutsche Zeitschrift für Nervenheilkunde* an interesting case of intracranial tumour in which unilateral visual hallucinations and macropsia formed the chief and localising symptoms. The patient was a woman who began gradually to suffer from intense headaches, situated in the occipital region. Some months later the acuity of her vision commenced to diminish, and not long thereafter giddiness and sickness developed. Her memory for recent events began to fail, and then definite visual hallucinations made their appearance. The patient said they took the form of stars, brown leaves, rings, &c., and that they were always localised in the right visual field. Many of these hallucinatory objects were noted by her to be very much larger than real objects of the same nature would have appeared. On examination it was found that there were double optic neuritis and right homonymous hemianopia. In addition, there was a slight degree of mind-blindness, and possibly also of alexia. It was also noted that the patient's ideas of distance and perspective were very much at fault. During the examination she frequently remarked: "The room seems so big," "The doctor seems so tall and his face so large." Not long thereafter the patient died, and at the necropsy a tumour was discovered in the occipital lobe of the left side, its posterior limit about one and a half centimetres from the occipital pole, extending forwards in the substance of the lobe to the lateral ventricle, into the wall of which it bulged. The anterior part of the lower margin of the calcarine fissure was invaded by tumour growth; the lingual gyrus was also invaded. Microscopically the tumour was a glioma. Among the interesting features of this case is the occurrence of macropsia. It is, of course, well known that macropsia and its opposite, micropsia, are found with certain peripheral lesions of the visual system. Paralysis of accommodation may produce the latter, accommodation spasm the former. The phenomena are by no means rare, further, as an aura in epilepsy, and in such cases, no doubt, the condition is of central origin. It has been remarked in hysteria, and there also

¹ Actinomycosis of Ox Tongues Imported from the Argentine Republic. Report of Medical Officers of Health for the City and Port of London. Nov. 13th, 1913.

² THE LANCET, Oct. 21st, 1911, p. 1152.

³ THE LANCET, vol. i., 1910, pp. 551, 626, 769.

the cause must be some central disturbance. Not a few organic cases have been recorded, either of micropsia or macropsia, where the lesion was certainly in the visual or visuo-psychic area of the cerebral cortex (Henschen, Uhthoff, Oppenheim, and others), and this case adds one more to the list. The exact localisation of the phenomenon, however, is by no means determined. Whether it is associated with the degree of mind-blindness from which the patient suffered is not proved. It is known that in that condition disturbances of stereoscopic vision may occur; in Oppenheim's case, however, micropsia was noted in the absence of any form of mind-blindness.

AN EARLY SYMPOSIUM ON VACCINATION.

IT is perfectly natural that in the opening years of the nineteenth century, when vaccination was still on its trial, there should have been numerous searchings of heart among old-fashioned medical men as to the efficacy of the new method. Vaccination at first, through being imperfectly understood by the operator, occasionally failed of its object, and we find a certain Dr. John Birch, of Spring Gardens, writing to the Royal College of Surgeons of England in 1807 to say that he believed the practice was decreasing in London, and citing at the same time a number of cases of so-called failure—i.e., the case of Rebecca Latchfield and those of Mr. Watts's children and the inmates of Lambeth Workhouse. But what really strikes the student of old reports on vaccination is the overwhelming unanimity of their writers in favour of Jenner's discovery. In 1806 a number of questions on vaccination were submitted to practitioners throughout the country, and were answered, tersely in most cases, but almost always in favour of what was still regarded as an experiment. The replies of Benjamin Brodie and John Abernethy are notable. The former, at that time assistant to Sir Everard Home, answers very briefly to the effect that he has vaccinated about 100 persons and has heard of no case of small-pox being caught by any of them subsequently. He knows of no case of ill-effects following vaccination. He says emphatically, "None whatever." His letter is dated from Home's house in Sackville-street (Dec. 28th, 1806). Abernethy, whose pupil Brodie had been, writes, using capitals in the eighteenth century manner. His replies are characteristic of the man.

1. How many Persons have you vaccinated? Abernethy: "I believe more than 200."

2. Have any of your Patients had the Small Pox after Vaccination?—in the Case of every such Occurrence, at what Period was the Vaccine Matter taken from the Vesicle?—how was it preserved? How long before it was inserted?—what was the Appearance of the Inflammation?—and what the Interval between Vaccination and the Variolous Eruption? Abernethy answers as follows: "Almost the whole of my Inoculations was done by way of ascertaining the Truth of Doctor Jenner's Assertions and Opinions and shortly after his first publication. I could not believe that the Action of so mild a poison as the vaccine could exempt the Constitution from the Influence of so potent a Contagion as the Small Pox; but though I tried by repeated inoculations and by exposing patients in Air which actually stank with variolous effluvia, I could never communicate that disease to persons previously vaccinated. It was upon the Ground of these Experiments, that I subscribed my name to a paper recommending vaccine inoculation to the public. Since that Time my Experience has been very limited as I have only inoculated the children of a few of my friends. I have not heard of any failure in these or former cases."

In answer to the question, "Have any bad effects occurred, in your Experience, in Consequence of Vaccination?—and, if so, what were they?" Abernethy merely answers, "None." "I do not know," he replies in answer to the fourth and last question, which is to this effect—"Is the Practice of Vaccination increasing, or decreasing, in your neighbourhood?—if decreasing, to what Cause do you impute it?" The document is signed "John Abernethy, Bedford Row, 31st Decr., 1806."

A TETANIC FORM OF URÆMIA.

VERY few cases have been published of a form of uræmia simulating tetanus, and these leave much to be desired. They belong to a period anterior to the use of modern methods of examination, such as lumbar puncture and estimation of the amount of urea in the blood and cerebro-spinal fluid. The rarity and unsatisfactory nature of the records have led Professor Chauffard to doubt the existence of this form of uræmia. Hence the importance of the following case reported at a meeting of the Société Médicale des Hôpitaux of Paris on Nov. 14th, 1913, by M. A. Bergé and M. Pernet. A single woman, aged 61 years, was taken to the Broussais Hospital on Sept. 10th, 1912, in a state of "generalised contracture" which had followed a fall. She was first admitted as a surgical case, but as the surgeon could find no wound and the accident appeared to have been trivial she was transferred to the medical side with the diagnosis of tetanus. The history was that she had been in good health until ten days previously, when she fell on the stairs. Eight days later she was attacked with trismus, slight dysphagia, and stiffness of the neck. On admission she was perfectly conscious, and was positive that she never previously suffered from any disease. On examination there was generalised rigidity, the head was retracted and buried in the pillow, the jaws were clenched, and the abdominal muscles were tense, giving to the abdomen a wooden feeling. The lower limbs were contracted in extension; the adductors stood out like tense cords. The upper limbs were slightly contracted in flexion. At intervals attacks of slight opisthotonos occurred. On attempting to flex the head the whole body was raised from the bed. There was no risus sardonicus. The eyes were kept constantly shut, but could be easily opened. The patient suffered no pain. The pupils were not contracted and reacted normally. The pulse ranged from 100 to 120, and the temperature from 98.6° to 100.4° F. There was incontinence of urine. The physician in charge of the case, in the absence of M. Bergé, diagnosed tetanus, and gave several injections of antitetanic serum, but the symptoms persisted. On Sept. 16th M. Bergé saw the patient for the first time and agreed with the diagnosis. She was then a little somnolent. On the 17th she became comatose and there was a remarkable change in the condition of the muscles. The lower limbs could easily be flexed and the abdomen was relatively supple, but there was slight rigidity of the upper limbs and the neck was still very rigid. The trismus had disappeared. The eyes were sunken and the conjunctivæ were almost insensitive. The pupils were not contracted and did not react to light. There was Cheyne-Stokes respiration with long pauses during which the patient appeared to be dead. The knee-jerks were normal and Babinski's sign was not present. The pulse was 100 and the temperature 101.3°. This great change in the patient's condition—the coma,